CHIEF COMPLAINT: Chronic, extensive red scaly skin plaques covering the scalp, elbows, knees, and trunk with pruritus and joint pain.

HISTORY: A 38-year-old male presents with a 10-year history of plaque psoriasis. Previous treatments with topical betamethasone dipropionate and calcipotriene ointment achieved only temporary partial clearance. Over the past six months, the lesions have expanded significantly. He reports morning stiffness lasting 45 minutes in his hands and swelling of his right second toe.

PHYSICAL EXAMINATION:

Dermatologic Examination:

- Scalp: Thick, confluent hyperkeratotic plaques with silvery-white micaceous scales extending beyond the hairline.

- Torso and Extremities: Symmetrical, well-demarcated erythematous plaques ranging from 3 to 12 cm with overlying silvery scales distributed over the extensor surfaces of the bilateral elbows, forearms, lumbosacral back, and anterior patellar knees. Auspitz sign (pinpoint bleeding upon gentle scale detachment) is positive.

- Nails: Prominent pitting (thimble-like indentations), subungual hyperkeratosis, and 'oil drop' dyschromic patches noted on eight fingernails.

- Musculoskeletal: Dactylitis ('sausage digit') of the right second toe with tenderness on palpation of the distal interphalangeal (DIP) joints of the hands.

Psoriasis Area and Severity Index (PASI) Score: Calculated at 18.6 with 22% total Body Surface Area (BSA) involvement.

ASSESSMENT AND MANAGEMENT:

1. Severe chronic plaque psoriasis with psoriatic nail dystrophy.

2. Psoriatic arthritis (dactylitis and DIP peripheral spondyloarthritis).

3. Candidate for targeted biologic therapy. Screen for tuberculosis with QuantiFERON-TB Gold, complete hepatitis B/C viral serologies, and baseline CBC and metabolic panel.

4. Plan initiation of IL-17 inhibitor (e.g., secukinumab) or IL-23 inhibitor (e.g., risankizumab) following negative infection screening.